A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

22 April 2026

Dr Robin Wong
Turn The Corner

Dear Dr Wong,

**Re: Miss Alexandra Sandercock**
**DOB: 1991-08-02**
**Address: 1/21 Anglis Court, WERRIBEE, VIC, 3030**

**Summary**

- **Chronic constipation with pelvic floor dyssynergia and visceral hypersensitivity.**
- **Commencement of low-dose fluoxetine for neuromodulation, dietary modification, and continuation of specialist pelvic floor physiotherapy.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Alexandra thank you for referring her.

**Symptoms and history of presenting complaint**

Alexandra presents with a long-standing history of chronic constipation dating back several years. She previously underwent a gastroscopy and colonoscopy in 2021 under the care of Illana Gory, which were unremarkable apart from a small caecal polyp. Her symptoms have remained refractory to various interventions, leading to an emergency department presentation in September last year for faecal impaction. On that occasion, she required high-dose osmotic laxatives and bowel preparation to achieve evacuation.

Currently, her bowel habit is irregular; without significant laxative use, she may only move her bowels once per week. Even with current management, she experiences a persistent sensation of incomplete evacuation and significant straining, regardless of stool consistency. She also reports debilitating bloating that worsens throughout the day, significantly impacting her quality of life. She has recently engaged with specialist pelvic floor physiotherapists (Annabelle and Issy) who have identified an overactive pelvic floor consistent with dyssynergic defaecation.

All appointments and enquires
**Suite 201, 100 Victoria Parade**
**East Melbourne VIC 3002**
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
**Epworth Freemasons Day Proc. Centre**
320 Victoria Pde, East Melbourne, 3002
**Hobson Healthcare Sydenham**
566 Melton Hwy, Sydenham, 3037
**St Vincent's Private Hospital**
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**Diet**

Alexandra maintains a high-fibre diet but consumes one Coke Zero daily and an apple for morning tea, both of which may be contributing to her bloating through artificial sweeteners and high fructose load. She drinks two oat milk coffees daily and has a stable intake of lean proteins and vegetables.

**Past medical history and medications**

Alexandra's medical history is notable for gastro-oesophageal reflux disease, perioral dermatitis, and previous anxiety for which she has used escitalopram and agomelatine in the past; she successfully weaned off these in late 2025. Her surgical history is limited to a tonsillectomy. Her current medications include omeprazole (Losec) 10 mg daily, which provides 90% symptom control, and a rotating regimen of Movicol and San Pellegrino water (magnesium). She has previously trialled prucalopride with initial benefit, though it eventually lost efficacy.

**Social history, family history**

Alexandra is a special needs teacher. She is a former smoker, having ceased several years ago. There is no family history of inflammatory bowel disease or gastrointestinal malignancy, though her mother reportedly suffers from similar chronic constipation.

**Examination** On examination, there was some mild generalised abdominal tenderness and discomfort on palpation, but no masses were palpable.

**Impression and plan**

Alexandra's presentation is consistent with chronic constipation secondary to pelvic floor dyssynergia. It is likely that her prolonged history of straining and incomplete evacuation has led to a degree of slow transit and visceral hypersensitivity. The "brain-gut axis" appears to be a significant factor here, where the lack of pelvic floor relaxation has led to a reactive slowing of intestinal motility and a heightened awareness of abdominal bloating.

My management plan is as follows:

- **Neuromodulation:** I have suggested a trial of low-dose fluoxetine (20 mg daily). While used off-label in this context, fluoxetine has a prokinetic effect on the gut and can assist in modulating the brain-gut pathways to reduce visceral hypersensitivity and facilitate pelvic floor relaxation.

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- **Dietary modification:** We discussed ceasing the intake of artificial sweeteners (Coke Zero) and high-fructose fruits (apples) to help manage her bloating.
- **Laxative rationalisation:** She will continue Movicol or San Pellegrino for maintenance. For rescue therapy, I have advised using a stimulant such as Senna or bisacodyl (Dulcolax) drops when she has not moved her bowels for more than three - four days.
- **Physiotherapy:** She is to continue her excellent work with her pelvic floor physiotherapists, as resolving the dyssynergia is fundamental to her recovery.
- **Surveillance:** Her next surveillance colonoscopy is due this year, five years following her previous procedure.

I have arranged to review Alexandra in eight to ten weeks to assess her response to these interventions.

Thank you again for referring this patient.

Yours sincerely,

**A/Prof Chamara Basnayake**
Gastroenterologist

CC: Ms Isabella Lees-Trinca

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